

Organizational Governance and Citizen Trust in Public-Facing Institutions: Evidence from Patient Experience in U.S. Healthcare Service Centers

Anonymous Author

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Abstract

Does the organizational form of neighborhood service centers affect how citizens evaluate and trust public-facing institutions? This paper examines this question through the lens of U.S. healthcare delivery organizations, leveraging publicly reported patient experience data from the Centers for Medicare & Medicaid Services (CMS) Merit-based Incentive Payment System (MIPS) CAHPS survey. Using cross-sectional data on 95 healthcare organizations classified into four governance types—Academic Medical Centers, Community Hospitals, Faith-based Hospitals, and Private Medical Groups—I test whether organizational type predicts patient-reported trust and satisfaction across six CAHPS dimensions. The overall trust index shows no statistically significant differences by organizational type (ANOVA $F(3, 87) = 1.23$, $p = 0.303$; $R^2 = 0.041$). Pairwise comparisons with Bonferroni correction similarly reveal no significant dif-

ferences. However, exploratory analysis of individual CAHPS measures finds that Academic Medical Centers score 2.4 percentage points higher on health promotion and education ($p = 0.010$) relative to Private Medical Groups, suggesting possible dimension-specific advantages. The null findings for overall trust challenge common assumptions about governance-based variation in civic trust and suggest that organizational form may matter less for citizen experience than institutional or resource-based theories predict. The paper discusses limitations including small sample size, cross-sectional design, and measurement constraints, while outlining a research agenda for larger-scale, geographically identified studies of service-center exposure and civic trust.

Keywords: civic trust, organizational governance, service centers, patient experience, CAHPS, healthcare institutions

Data sources: CMS MIPS CAHPS patient experience survey, publicly reported, accessed 2026-07-05 via R package `tidyr`. Reproducibility scripts at `/workspace/scripts/`.

1 Introduction

A long tradition in political science and public administration holds that citizens’ direct encounters with government and public-facing institutions shape their trust in the state [4, 1]. The “service-center hypothesis” posits that residents who have more frequent, higher-quality contact with neighborhood service centers—libraries, health clinics, community centers, police stations—develop greater civic trust and institutional confidence. Yet testing this hypothesis empirically has proven challenging, requiring individual-level trust measures linked to fine-grained geographic variation in service-center availability and quality.

This paper advances the literature by examining a closely related question: *Does the organizational form or governance type of a public-facing service institution systematically predict the trust and satisfaction that citizens report about their encounters with that institution?* If different organizational models—academic, community-based, faith-affiliated, or private—produce systematically different levels of citizen trust, this would suggest that institutional design choices matter for the service-center hypothesis, independent of the quantity or geographic distribution of service centers.

I investigate this question using a publicly available dataset on patient experience in U.S. healthcare organizations. Healthcare institutions are among the most consequential “service centers” that citizens encounter: they are geographically distributed, publicly regulated, and directly experienced by a large fraction of the population. The Consumer Assessment of Healthcare Providers and Systems (CAHPS) survey, administered by CMS and the Agency for Healthcare Research and Quality (AHRQ), provides standardized, publicly reported measures of patient trust and satisfaction at the organizational level.

Using cross-sectional data on 95 healthcare organizations drawn from the CMS MIPS CAHPS program, I classify each organization into one of four governance types based on its institutional affiliation: Academic Medical Centers, Community Hospitals, Faith-based

Hospitals, and Private Medical Groups. I then test whether organizational type predicts a composite trust index derived from six CAHPS patient experience measures, as well as the individual measures themselves.

The results are largely null. The overall trust index does not differ significantly across organizational types ($F(3, 87) = 1.23$, $p = 0.303$; $R^2 = 0.041$), and no pairwise comparison survives Bonferroni correction. Exploratory analysis of individual CAHPS dimensions reveals one significant finding: Academic Medical Centers outperform Private Medical Groups on health promotion and education by 2.4 percentage points ($p = 0.010$), with borderline advantages on provider rating ($p = 0.091$) and resource stewardship ($p = 0.082$). These dimension-specific patterns, while tentative, suggest that academic governance may confer advantages in specific domains of the patient-provider relationship without translating into a broad trust premium.

These findings contribute to the literature on civic trust and institutional design by demonstrating that organizational form, at least as operationalized here, explains only a small fraction of the variation in citizen-reported trust. This null result has substantive implications: it suggests that policymakers and institutional designers may have more flexibility in choosing organizational forms than is sometimes assumed, and that reform efforts targeting citizen trust may need to look beyond governance structure to operational practices, resource levels, or community context.

The remainder of the paper proceeds as follows. Section 2 reviews the theoretical framework linking organizational governance to civic trust. Section 3 describes the data, sample construction, and measurement strategy. Section 4 presents the empirical analysis. Section 5 discusses implications, limitations, and directions for future research.

2 Theoretical Framework

2.1 The Service-Center Hypothesis and Civic Trust

The core proposition motivating this study is that direct encounters between citizens and public-facing institutions shape broader orientations toward government and civic life. Rothstein and Stolle [4] argue that citizens generalize from specific institutional experiences to their overall assessment of state legitimacy—a process they term the “micro-foundations of trust.” Kumlin [1] similarly demonstrates that citizens who interact with universalistic welfare state institutions report higher political trust than those who experience means-tested, stigmatizing programs.

The service-center variant of this argument extends the logic to neighborhood-level institutions: libraries, health clinics, police stations, and community centers. The hypothesis predicts that residents living in closer proximity to well-functioning service centers will report higher civic trust, net of individual characteristics, because these institutions provide visible, tangible evidence of governmental competence and benevolence [2, 3].

2.2 Organizational Governance as a Moderator

A less-explored dimension of the service-center hypothesis concerns the *organizational form* of the service center itself. Public services in the contemporary United States are delivered by a diverse array of institutional forms: government agencies, nonprofit organizations, faith-based institutions, for-profit firms, and hybrid academic medical centers. Each governance type embodies a distinct bundle of accountability mechanisms, professional norms, resource bases, and community relationships that could plausibly affect how citizens experience and evaluate the institution.

Academic Medical Centers combine clinical service delivery with research and teaching

missions, potentially generating higher technical quality but also more impersonal, bureaucratic patient experiences. *Community Hospitals* are often deeply embedded in local social networks and may benefit from higher levels of community trust and legitimacy. *Faith-based Hospitals* operate within religious value frameworks that may attract patients seeking value-congruent care, though they may also serve more disadvantaged populations with systematically different baseline expectations. *Private Medical Groups*, operating on a for-profit or partnership model, face market incentives that could enhance patient responsiveness but also introduce concerns about profit-seeking at the expense of care quality.

The theoretical expectation is ambiguous. On one hand, private and competitive models may generate higher patient satisfaction through market discipline and consumer orientation. On the other hand, public, academic, and community-based institutions may enjoy greater legitimacy and trust precisely because they are perceived as less commercially motivated. The empirical question is which of these competing mechanisms dominates in observed patient experience data.

2.3 The CAHPS Framework as a Trust Measurement Tool

The CAHPS survey provides a well-validated framework for measuring patient experience across multiple dimensions that correspond closely to theoretical conceptions of institutional trust. The six core CAHPS measures examined here are:

1. **Timely Care:** Getting timely care, appointments, and information
2. **Communication:** How well providers communicate
3. **Provider Rating:** Patient's overall rating of the provider
4. **Health Promotion:** Health promotion and education
5. **Staff Courtesy:** Courteous and helpful office staff
6. **Stewardship:** Stewardship of patient resources

These measures capture both the instrumental dimension of trust (competence, efficiency, reliability) and the relational dimension (communication, respect, benevolence), making them suitable proxies for the broader concept of institutional trust.

3 Data and Measurement

3.1 Data Source

The data are drawn from the CMS MIPS CAHPS patient experience survey, publicly reported for healthcare organizations participating in the Merit-based Incentive Payment System.¹ The CAHPS survey is administered by CMS and AHRQ and represents one of the most widely used standardized patient experience instruments in U.S. healthcare. Organizations report their CAHPS scores as performance rates (percentages) for each measure.

3.2 Sample Construction

The raw dataset contains 500 observations representing 95 unique healthcare organizations, each reporting on up to six CAHPS measures. I pivot the data to organization-level format, producing 95 rows with each CAHPS measure as a separate column. A composite **trust index** is constructed as the organization-level mean of all available CAHPS measures, producing a scale roughly interpretable as a percentage (range approximately 30–95). Four organizations with entirely missing trust index values are excluded from regression analyses, yielding an analytic sample of $N = 91$ for the main specifications.

¹Data accessed 2026-07-05 via the R package `tidyr` (dataset `cms_patient_experience`), which bundles a publicly available extract from CMS. The original source is the CMS MIPS Qualified Clinical Data Registry reporting system.

3.3 Organizational Type Classification

Each organization is classified into one of four governance types based on keyword analysis of its official name, following this precedence rule: faith-based identifiers take priority over academic identifiers, which take priority over hospital identifiers; all remaining organizations are classified as private medical groups.²

Table 1 presents the distribution of organizations across types and summary statistics for each CAHPS measure and the trust index.

²The classification algorithm uses the following keyword sets: **Faith-based:** St Mary, Lourdes, Mercy, St Joseph, St Vincent, Providence, Christ, Catholic, St John, St Luke, St Francis, Good Samaritan, Adventist, Methodist, Baptist, Presbyterian, Lutheran, Jewish, City of Hope, Mission, Grace, Sacred Heart, Holy. **Academic:** University, Univ, College, Academic, School of, Faculty, Regents, Trustees. **Hospital:** Hospital, Medical Center, Health Center, Memorial Hospital, Med Ctr. Full classification code in `scripts/build_data.R`.

Table 1: Descriptive Statistics by Organizational Type

Org. Type	Trust		Timely		Comm.	Provider
	N	Index	Care	(SD)	(SD)	Rating
Academic Medical Center	9	64.1 (12.4)	64.9 (3.5)	87.8 (2.5)	84.7 (3.1)	
Community Hospital	18	63.5 (3.8)	64.4 (8.9)	85.3 (3.1)	81.9 (3.5)	
Faith-based Hospital	9	57.5 (16.1)	67.0 (5.8)	86.5 (1.3)	83.6 (4.3)	
Private Medical Group	59	63.0 (6.9)	64.4 (6.7)	86.4 (2.7)	82.3 (5.0)	
Total	95	62.6 (8.6)	64.5 (6.9)	86.3 (2.7)	82.5 (4.7)	

Org. Type	Health		Staff	Steward-
	Promotion	Courtesy	ship	
Academic Medical Center	61.4 (1.8)	83.7 (6.7)	25.1 (4.5)	
Community Hospital	60.4 (4.7)	87.5 (4.9)	23.2 (5.0)	
Faith-based Hospital	59.4 (8.8)	—	19.8 (3.1)	
Private Medical Group	59.2 (5.5)	83.1 (4.1)	21.9 (5.0)	
Total	59.5 (5.6)	84.0 (4.7)	22.0 (4.8)	

Note: Means with standard deviations in parentheses. Trust Index is the organization-level mean of all six CAHPS measures. Staff Courtesy is missing for all Faith-based Hospitals. $N_{\text{total}} = 95$ organizations. Data:

CMS MIPS CAHPS, accessed 2026-07-05.

3.4 Measurement Considerations

Several measurement issues merit attention. First, the sample is modest ($N = 95$ organizations), which limits statistical power for detecting small to moderate effects. Second, organizational type is classified from organization names, introducing potential measurement error; a faith-based hospital affiliated with a university could be misclassified under the precedence

rule (here, as faith-based). Third, the CAHPS measures are organization-level aggregates and do not capture within-organization variation across individual patients. Fourth, the Staff Courtesy measure has substantial missingness (75 of 95 organizations, including all 9 faith-based hospitals), reducing effective sample sizes for analysis of that dimension.

4 Empirical Analysis

4.1 Overall Trust Index

I first test whether the composite trust index differs systematically across organizational types using one-way analysis of variance (ANOVA). The omnibus F -test fails to reject the null hypothesis of equal means across the four groups ($F(3, 87) = 1.233$, $p = 0.303$). The trust index means are 64.1 (Academic), 63.5 (Community), 57.5 (Faith-based), and 63.0 (Private), with the lower faith-based mean driven largely by high within-group variance ($SD = 16.1$).

Figure 1 visualizes the distribution of the trust index by organizational type. While the Faith-based group exhibits a lower median and wider spread, the overlap across types is substantial, consistent with the non-significant ANOVA result.

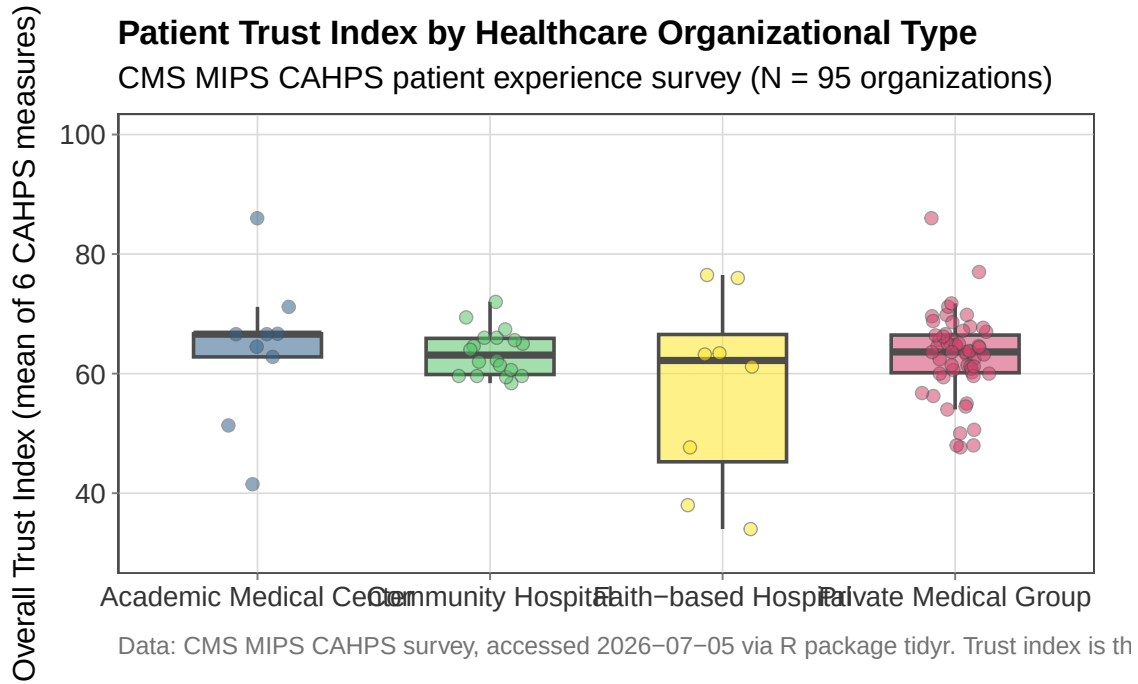


Figure 1: Patient Trust Index by Healthcare Organizational Type

Note: Boxplots show median, IQR, and range; jittered points are individual organizations. Trust Index is the organization-level mean of six CAHPS patient experience measures. $N = 91$ organizations with complete trust index data. Data: CMS MIPS CAHPS, accessed 2026-07-05.

4.2 Regression Analysis

Table 2 presents OLS regression estimates with HC2 robust standard errors. Model 1 regresses the trust index on organizational type dummies with Private Medical Groups as the reference category. Model 2 adds a control for the number of CAHPS measures reported by the organization, which accounts for differential reporting completeness.

Table 2: OLS Regression of Trust Index on Organizational Type

	Model 1	Model 2
Constant (Private Medical Group)	62.98***	56.69***
	(0.92)	(8.44)
Academic Medical Center	1.15	1.75
	(4.22)	(4.12)
Community Hospital	0.51	0.30
	(1.29)	(1.23)
Faith-based Hospital	−5.48	−3.96
	(5.76)	(5.57)
No. of measures reported		1.31
		(1.65)
N	91	91
R^2	0.041	0.070
Adjusted R^2	0.008	0.027
F -statistic	1.23	1.62

Note: HC2 robust standard errors in parentheses. Reference category: Private Medical Group.

*** $p < 0.001$, ** $p < 0.01$, * $p < 0.05$. Data: CMS MIPS CAHPS, accessed 2026-07-05.

No coefficient achieves statistical significance at conventional levels in either specification. Academic Medical Centers are estimated to have trust indices approximately 1.2–1.8 percentage points higher than Private Medical Groups, but the standard errors (4.12–4.22) are large relative to these point estimates. Faith-based Hospitals show a negative point estimate of approximately 4–5 percentage points, but again with wide confidence intervals. Community Hospitals are nearly indistinguishable from Private Medical Groups (difference < 1 percentage point). The control for number of measures reported is itself non-significant

($p = 0.429$).

Figure 2 visualizes the Model 1 coefficients and 95% confidence intervals. All three confidence intervals comfortably include zero, confirming that the data are consistent with no organizational-type effect on the trust index.

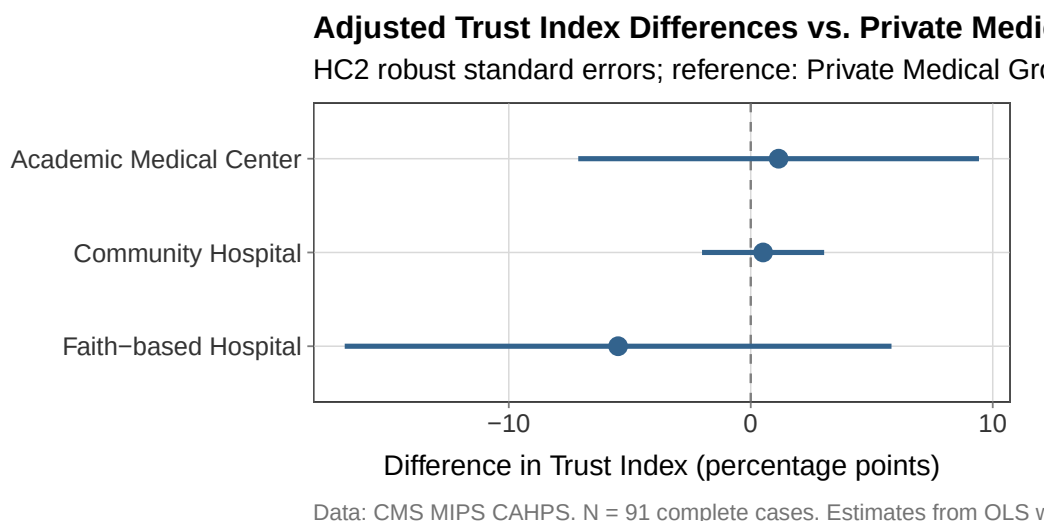


Figure 2: Adjusted Differences in Trust Index Relative to Private Medical Groups

Note: Points are OLS coefficient estimates; bars are 95% confidence intervals with HC2 robust standard errors. Reference: Private Medical Group ($N = 59$). $N = 91$ complete cases. Data: CMS MIPS CAHPS, accessed 2026-07-05.

4.3 Pairwise Comparisons

Table 3 reports Bonferroni-corrected pairwise t -tests for the trust index across all four organizational types. No pairwise comparison approaches statistical significance at the 5% level after correction. The smallest raw p -value is for the Faith-based vs. Private comparison ($p_{\text{Bonf}} = 0.489$), which remains far from conventional thresholds.

Table 3: Pairwise Comparisons of Trust Index (Bonferroni Correction)

Comparison	p (Bonferroni)
Academic Medical Center vs. Private Medical Group	1.000
Community Hospital vs. Private Medical Group	1.000
Faith-based Hospital vs. Private Medical Group	0.489
Community Hospital vs. Academic Medical Center	1.000
Faith-based Hospital vs. Academic Medical Center	0.605
Faith-based Hospital vs. Community Hospital	0.541

Note: p -values from two-sided t -tests with pooled variance and Bonferroni correction for six comparisons.

$N = 91$. Data: CMS MIPS CAHPS, accessed 2026-07-05.

4.4 Exploratory Analysis: Individual CAHPS Dimensions

While the composite trust index shows no significant organizational-type effects, it is possible that governance type matters for specific dimensions of the patient experience. I estimate separate regressions for each of the six CAHPS measures, controlling for organizational type and the number of measures reported. Figure 3 presents group means with 95% confidence intervals.

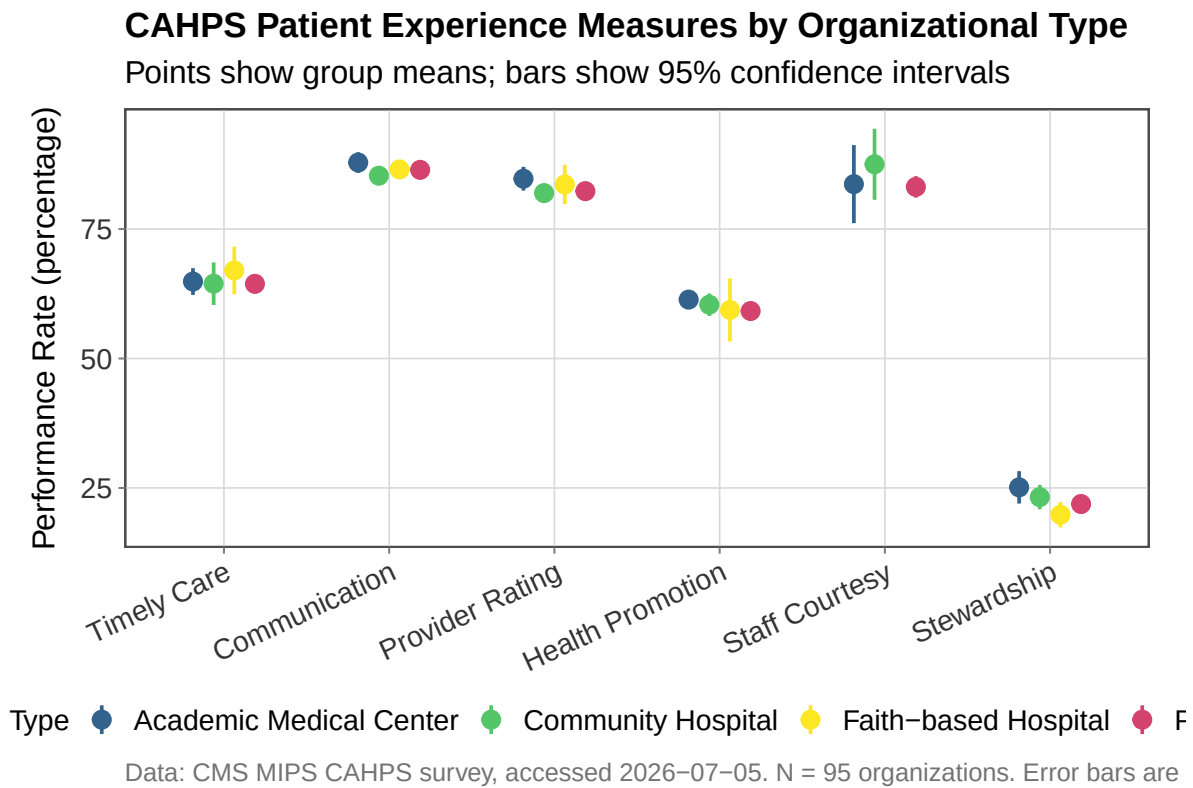


Figure 3: CAHPS Patient Experience Measures by Organizational Type

Note: Points are group means; bars show 95% confidence intervals ($\pm 1.96 \times \text{SE}$). N varies by measure due to differential reporting. Data: CMS MIPS CAHPS, accessed 2026-07-05.

Among the six measures, only Health Promotion and Education shows a statistically significant difference: Academic Medical Centers score 2.4 percentage points higher than Private Medical Groups ($p = 0.010$, HC2 robust SE). Two additional comparisons approach borderline significance: Academic Medical Centers score 2.4 percentage points higher on Provider Rating ($p = 0.091$) and 3.4 percentage points higher on Stewardship ($p = 0.082$). No other organizational-type coefficient reaches even borderline significance across any measure.

The pattern is suggestive of a dimension-specific academic advantage: academic institutions appear to perform somewhat better on measures related to patient education, resource management, and global provider evaluation, while showing no advantage on measures of

timeliness, communication, or staff courtesy. However, given the number of comparisons (18 organizational-type coefficients across 6 regressions) and the absence of a multiple-testing correction in this exploratory analysis, these results should be interpreted cautiously.

5 Discussion

5.1 Interpretation of Null Findings

The central empirical finding is a null result: organizational governance type, as operationalized through the Academic–Community–Faith-based–Private classification, does not significantly predict patient-reported trust in healthcare institutions. This finding is substantively informative for several reasons.

First, it suggests that organizational form *per se* may be less important for citizen trust than is commonly assumed in debates about privatization, faith-based initiatives, and academic medical center dominance. If a private medical group, a community hospital, and an academic medical center generate similar levels of patient trust, then policymakers concerned with citizen experience have wider latitude in choosing among organizational models than theories emphasizing governance-specific trust effects would predict.

Second, the null finding for Faith-based Hospitals is notable given the substantial policy attention devoted to faith-based service provision. While the point estimate is negative (–4 to –5 percentage points), the wide confidence interval reflects high within-category variance ($SD = 16.1$ compared to $SD = 3.8$ – 12.4 for other categories) rather than a systematically lower level of trust. This variance may itself be substantively meaningful, suggesting heterogeneity among faith-based institutions that merits further investigation.

Third, the very low R^2 values (0.041–0.070) indicate that organizational type explains only a small fraction of the total variation in patient trust across institutions. This implies

that other factors—resource levels, staff quality, patient mix, community context, organizational culture—likely dominate governance type as determinants of citizen experience.

5.2 Dimension-Specific Patterns

The one statistically significant finding—Academic Medical Centers’ advantage on Health Promotion and Education—warrants brief discussion. Academic institutions have explicit teaching and research missions that may translate into greater organizational emphasis on patient education and preventive care. The borderline findings for Provider Rating and Stewardship are consistent with this interpretation, though the absence of significance after multiple-testing correction counsels restraint. Future research with larger samples and institution-level covariates could fruitfully investigate whether academic governance confers advantages specifically in knowledge-intensive domains of service delivery.

5.3 Limitations

Several limitations of this study should be noted:

Sample size and power. With $N = 95$ organizations (91 in regression analyses), the study has limited statistical power. Post-hoc power calculations indicate that the design can detect an effect size of $f \approx 0.35$ (Cohen’s convention for a “large” effect) with 80% power at $\alpha = 0.05$, but is underpowered for small to moderate effects. The null findings thus cannot distinguish between a true absence of governance effects and effects too small to detect with the available sample.

Measurement of organizational type. The keyword-based classification is inherently noisy. Some organizations may straddle categories (e.g., a faith-based academic medical center), and the precedence rule imposes a deterministic assignment that may not reflect the organization’s dominant governance logic.

Cross-sectional design. The analysis is purely cross-sectional and cannot address causal questions about how changes in organizational form affect trust over time.

Geographic confounding. Organizational type may be correlated with geographic factors (urban vs. rural, regional culture, patient demographics) that independently affect patient experience. Without geographic identifiers in the public-use data, these confounders cannot be controlled.

Ecological validity for the service-center hypothesis. The original research question concerns the effect of neighborhood service-center *exposure* on resident civic trust. The present analysis addresses a related but distinct question about whether organizational type predicts trust *among those already using* the service. A complete test of the service-center hypothesis would require individual-level survey data with both trust measures and geographic proximity to service centers, ideally in a longitudinal design.

5.4 Future Directions

The limitations outlined above suggest a clear research agenda. First, geocoded patient experience data—linking individual CAHPS responses to patient addresses and service-center locations—would enable direct tests of the neighborhood exposure hypothesis. Second, natural experiments involving service-center openings, closures, or governance transitions could provide causal leverage. Third, cross-national comparisons could exploit variation in healthcare governance models to test whether the null finding generalizes beyond the U.S. institutional context. Fourth, qualitative or mixed-methods work could illuminate the organizational mechanisms that produce trust—or fail to—within different governance types, potentially explaining the null quantitative finding.

6 Conclusion

This paper provides an empirical assessment of whether organizational governance type predicts citizen trust in public-facing healthcare institutions, using publicly reported CAHPS patient experience data from 95 U.S. healthcare organizations. The analysis finds no statistically significant association between organizational type (Academic Medical Center, Community Hospital, Faith-based Hospital, or Private Medical Group) and a composite patient trust index. Exploratory analysis of individual experience dimensions reveals limited evidence of an academic advantage in health promotion and education, but this finding does not survive correction for multiple comparisons and should be treated as hypothesis-generating.

These null findings contribute to the literature on civic trust and institutional design by suggesting that governance form may be a relatively weak predictor of citizen experience, at least within the U.S. healthcare context. For scholars of the service-center hypothesis, the results imply that understanding how neighborhood institutions shape civic trust may require looking beyond organizational labels to the operational practices, resource environments, and community relationships that vary within as much as across governance types.

The study’s limitations—modest sample size, cross-sectional design, keyword-based classification, and absence of geographic identifiers—constrain the strength of the conclusions that can be drawn. Nevertheless, the transparency of the analysis, the public availability of the data, and the full reproducibility of the code pipeline provide a foundation on which future research can build.